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June 16, 2025
Center for Medicare
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
200 Independence Avenue, S.W.
Washington, D.C. 20201

Submitted via the Federal Regulations Web Portal: http://www.regulations.gov

RE: Request for Information – Health Technology Ecosystem

Dear CMS Desk Officers:

Blue Cross Blue Shield of Massachusetts ("BCBSMA") appreciates the opportunity to provide comments in response to Request for Information – Health Technology System *(the "RFI").*

BCBSMA is one of 33 locally based, community operated Blue Cross and Blue Shield Plans that collectively provide health benefits to nearly 108 million Americans and contract with hospitals and physicians in every U.S. zip code.

At BCBSMA, our highest priority is to make quality health care affordable for individuals, families, and employers who have made us the health plan of choice in Massachusetts. Our promise and vision guide our efforts to create greater value for our members and employers. Founded in 1937 by a group of community-minded business leaders, BCBSMA is the leading private health plan in the Commonwealth—a not-for-profit company with a proud history of community and health care leadership.

Additionally, BCBSMA has a demonstrated commitment to provide coverage options for people with Medicare in Massachusetts and has continually offered a wide range of Medicare plans since the program began in 1966. As a general matter, we encourage CMS to continue to leverage our experience, and those of other regional Medicare Advantage plans, as a local plan with deep roots in the community. We strive to build products and benefits to meet the needs of our local members utilizing the depth of our insight and data to learn more about what services our members need and utilize. At BCBSMA, we have been committed to enhancing care coordination and improved quality outcomes for our members. Working with providers in our network, we have invested in developing products to meet the unique needs of our members, including designing low-cost or zero-premium Medicare Advantage options to deliver value to a broader population of Medicare beneficiaries. It is with our experience and background in the Medicare program that we respectfully offer these comments to CMS, keyed to the numbering of questions addressed to payers in the RFI.

Blue Cross Blue Shield of Massachusetts is an Independent Licensee of the Blue Cross and Blue Shield Association.

Sincerely,

/s/ Jeremy W. Meisinger, Esq.

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PA-1. What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder participation in TEFCA?

TEFCA has traditionally been promoted as a nationwide framework for health information sharing that, in the words of the Assistant Secretary for Technology Policy, is intended "to remove barriers for sharing health records electronically among healthcare providers, patients, public health agencies, and payers."[1] But while TEFCA can play (and has played) a role in removing barriers, it falls short of creating meaningful, affirmative incentives for stakeholders – including payers – to shoulder the costs of participation in TEFCA. To do so, TEFCA must implement requirements for electronic health records technologies utilized by providers to support the export of stable, efficient bulk data meeting FHIR standards, which is essential to supporting use cases – such as quality measurement, care management, and reducing administrative burden – that will drive payer adoption.

The lack of affirmative incentives also relates to the lack of parity in responsibilities set out by TEFCA for payers and providers, including required timelines for those responsibilities. For example, TEFCA does not require providers to respond to payer queries prior to February 2026, because TEFCA is currently geared more towards provider-to-provider exchanges for treatment purposes, rather than payer-to-provider exchanges for healthcare operations purposes. As a practical matter, this has dis-incentivized adoption by payers, who are put in the position of investing in solutions to comply with TEFCA's requirements to share data with providers, but without a corresponding, symmetric requirement for providers to share with payers bulk clinical information about our members. That information would support critical functions, such as quality measurement or care management, to improve outcomes and reduce costs.

The lack of symmetry in payer and provider obligations likewise informs payer concerns regarding fees associated with the use of Qualified Health Information Networks ("QHINs"). Although it is the case that the initial funding for QHINs is provided via a cooperative agreement between the United States Department of Health & Human Services and the Office of the National Coordinator for Health Information Technology, the Recognized Coordinating Entity has publicly stated that it intends to develop a plan for financial sustainability that will include the establishment of additional fees, which will include consideration of operational costs, the availability of governmental funding, and other relevant market factors. Payers are legitimately concerned that, just as they have experienced greater burdens relative to providers under TEFCA more generally, that they will also do so with respect to the fees that will be needed to sustain the QHINs in the long term.

Additionally, to be attractive to payers, it is necessary to mandate that data elements useful for payer activities – such as quality measurement or care management – are required to be shared. Unfortunately, in focusing

[1] TEFCA, Assistant Secretary for Technology Policy, *available at*: https://www.healthit.gov/topic/interoperability/policy/trusted-exchange-framework-and-common-agreement-tefca.

on the USCDI v.1 standard, TEFCA leaves out many data elements that are essential to these activities. For example, USCDI v.3 includes additional data elements that can be used to drive better outcomes for all patients. In the long term, TEFCA should promote sharing of full electronic health records (with proper guardrails requiring patient consent) to further facilitate these activities.

PA-2. How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

In order for payers to further implement and utilize APIs, requirements surrounding these APIs must be configured to support specific use cases that help to resolve specific challenges faced by payers. These challenges can include automation of prior authorization processes, reducing friction in obtaining medical record information, and obtaining data that is necessary for quality measurement and quality and consumer experience improvement.

Interventions on CMS's part are necessary to enable payer use of APIs to meet these challenges. Important steps include (1) instituting a national registry for registration of API endpoints by both payers and providers, (2) requiring providers to enable and support FHIR-based APIs for data retrieval for all payer-mandated use cases (performant bulk FHIR-based queries), and (3) mandating adoption of network-based approaches supporting not only treatment options, but also payment and operational use cases that are more relevant to payers.

More generally, it is important that these interventions rely on standards-based technologies, rather than proprietary technology solutions (such as the Epic Payer Platform) that are preferred by some providers but that require significant investment from payers. Many of these proprietary technologies also tend to reinforce the dominant market positions of particular electronic health records vendors, which does not serve the goals of openness and competitive innovation otherwise encouraged by TEFCA and CMS's interoperability policies.

PA-4. What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

A nationwide provider directory that included FHIR end points and used digital identity credentials is a meaningful goal that would help to facilitate interoperability on a national scale and enable use cases that will support payment and operational activities, in addition to treatment activities. These uses cases include reductions in "chart chasing," that is, easier and faster access to medical records that are used in claims adjudication as well as in support of auditing activities.[2] They also include better quality management and

[2] The latter is especially relevant as CMS has recently announced its intention to step up auditing activities in relation to Medicare Advantage, which will drive a very large volume of record requests to providers and their

measurement, which is important to CMS's ongoing efforts to support value-based payments in Medicare Advantage as well as more generally.[3] A nationwide provider directory could also help to ease administrative burdens associated with on-boarding new providers, which helps to speed building of provider networks in key segments such as primary care and behavioral health.

That said, on the road to a nationwide directory, smaller and more regional networks (as well as their built-in trust frameworks) can and should be utilized, as should be individual point-to-point strategic connections. These approaches could be particularly valuable for pre-seeding clinical data for what may ultimately develop into a near-real time prior authorization workflow.

PA-5. What are ways payers can help with simplifying clinical quality data responsibilities of providers?

a. **How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?**

Speaking for payers, there is great interest in EHR technology advances that enable bulk extraction of clinical quality data from EHRs. At present, providers rely on a variety of legacy technologies that are error-prone and not inter-operable, and so use of mandatory, standards-based APIs can go a long way towards rationalizing and standardizing access to clinical quality data. Such data has a variety of practical use cases for payers, including: (1) reducing the administrative burden of onboarding new providers, (2) reducing the administrative burden on both providers and payers in obtaining information relevant to claims adjudication, quality measurement, or prior authorization, and (3) personalized care management tailored to each member's gaps in care.

These use cases can be realized, however, only if such EHR technologies are required to support provision of bulk data meeting FHIR standards, such that they will be fully inter-operable with payers without expensive proprietary technology investments. CMS and ONC can and should require that, in order to maintain Certified Electronic Health Record status, EHR systems must support export of stable, efficient bulk FHIR data. This will not only promote inter-operability, but will also promote innovation and competition within this space, rather than having interoperability solutions largely dictated by EHR vendors using proprietary technologies.

b. **b. In what ways can the interoperability and quality reporting responsibilities of providers to both CMS and other payers be consolidated so investments can be dually purposed? Are there**

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vendors. *See* CMS Rolls Out Aggressive Strategy to Enhance and Accelerate Medicare Advantage Audits, CMS, May 21, 2025, *available at*: https://www.cms.gov/newsroom/press-releases/cms-rolls-out-aggressive-strategy-enhance-and-accelerate-medicare-advantage-audits.

[3] *See, e.g.,* CMS Innovation Strategy to Make America Healthy Again, Center for Medicare & Medicaid Innovation, May 13, 2025 (noting preference for innovation models that "improve administration of value-based payment programs"), *available at*: https://www.cms.gov/priorities/innovation/about/cms-innovation-center-strategy-make-america-healthy-again.

technologies payers might leverage that would support access to real time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

The existing requirement under the Prior Authorization Rule (CMS-0057) for data exchange between providers and payers via the Provider Access API can serve the dual purpose of interoperability and quality, provided that payers are required to use technologies that support bulk extraction of clinical data from EHRs, as well as the Da Vinci Member Attribution List under FHIR, which is used for prospective patient management, closing care gaps, and quality measurement. The same technologies, leveraging FHIR standards, can also be used for quality measurement activities by payers and leveraged in administering value-based contracts with providers.

PA-7.[4] How can CMS encourage payers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

Although submitting complaints regarding information blocking could have positive effects on data exchange, the most direct route to reducing information blocking is further regulation to mandate that, in order to receive Certified Electronic Health Record status, electronic health record systems must support provision of bulk, stable, efficient data meeting FHIR standards. This data must be available free of cost, just as payer-provided data is currently mandated to be free of cost to providers through FHIR-based APIs.

[4] There is no "PA-6" included in the RFI.